Authoritative Sources for Each Research Question

Companion to the India Healthcare Sector Scan (Q1 to Q10) | 31 October 2026

How to use this map

- Rows are ordered by your priority ranking, from Indian government and regulators (1) to financial press (5), within each research question. Items marked "confirm" are things we expect to exist but have not verified.
- We name organizations and document types. Links in the third column and in the link register at the end go to each organization's home page, not to individual reports. We deliberately did not add deep links to specific documents, because we could not open them to confirm they exist or are current. Search each site for the document named in column four. No figures are quoted.
- Link check: the links come from our knowledge of official sites and were not opened in this session (network access was blocked). Click each before relying on it, and replace any that have moved. Where an organization's own site could not be confirmed (for example the Data Protection Board of India), no link is given; use the MeitY site until it is confirmed.
- Primary means original data, official statistics, legal texts or the company's own filings and records. Secondary means analysis or compilation of those. Company-reported data is primary for what the company reports but still needs independent verification when it is the target.
- Official Indian statistics often lag by years. Pair them with filings and administrative data for current positions, and say so in the investment case.

Topic coverage

Topic you asked for	Questions	Main evidence
Market size and growth	Q1, Q2, Q9	NHA, NSS, DPIIT, IRDAI, company filings; WHO and World Bank for benchmarks
Demand drivers	Q1, Q5	NFHS, NSS, PM-JAY and IRDAI data, census projections, ICMR disease studies
Technology and digital health adoption	Q5	ABDM dashboards and specifications, telemedicine guidelines, company IT disclosures, WHO and standards bodies
AI adoption	Q6, Q5	Target AI register and licences, IndiaAI / MeitY, ICMR, peer-reviewed validation, patent records
Competitive landscape	Q2	CCI orders, exchange filings, MCA data, rating agencies, deal databases
Regulatory environment	Q3, Q4, Q10	CDSCO, NPPA, state regulators, DPIIT, RBI, Gazette; foreign regulators for exporters
M&A and investment activity	Q2, Q9	SEBI offer documents, Ind AS 103 notes, CCI, DPIIT; IVCA and Bain for context
Key risks	Q3, Q6 to Q9	Regulator records, court orders, company risk disclosures, quality-of-earnings work
Future trends	Q10, Q5, Q6	Draft notifications, Parliamentary reports, WHO, OECD, peer-reviewed research
AI governance and responsible AI	Q6	ICMR 2023, MeitY / IndiaAI, NITI Aayog, CDSCO, WHO, OECD, ISO / IEC, EU AI Act, FDA
Data governance, privacy and cybersecurity	Q7	DPDP Act and Rules 2025 (Gazette), CERT-In, ABDM policy, SPDI Rules during transition, company incident records

Source map by research question

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q1. Market size, growth and demand drivers by subsector and geography (sector-level with target-specific capture)					
Q1	1. Government / regulator / statutory body	MoHFW / National Health Systems Resource Centre; Central Bureau of Health Intelligence	National Health Accounts estimates series; National Health Profile; Rural Health Statistics (confirm latest edition of each)	Total, government and out-of-pocket health expenditure and their shares; spending by provider type; bed, facility and workforce counts by state. Official statistics with known publication lag, so use for structure and history, not for current-year size.	Primary: original official statistics produced by the government.
Q1	1. Government / regulator / statutory body	MoSPI / National Statistical Office; IIPS and MoHFW; Registrar General of India	NSS health and morbidity / hospitalisation survey rounds; household consumption expenditure survey; Annual Survey of Industries; National Accounts Statistics; NFHS; Sample Registration System; Census-based projections	Utilisation, out-of-pocket spend, share of public versus private care, insurance coverage, disease prevalence, manufacturing output, GVA and employment of pharma and device industries. Statistically designed surveys; confirm which round is current.	Primary: survey microdata and official tables.
Q1	1. Government / regulator / statutory body	Department of Pharmaceuticals; Department of Commerce (DGCI&S); Pharmexcil and other export promotion councils	Departmental annual reports; PLI scheme and bulk drug / device park documents; foreign trade statistics by HS code	Domestic production, exports and imports of formulations, APIs, key starting materials and devices; import dependence by product and country. Customs-based trade data is highly reliable.	Primary: administrative and customs data.
Q1	1. Government / regulator / statutory body	National Health Authority; ESIC; CGHS (MoHFW); IRDAI	PM-JAY dashboards and annual reports; CGHS and ESIC annual reports; IRDAI Annual Report and Handbook on Indian Insurance Statistics	Beneficiaries, empanelled hospitals, claim counts and values by state and specialty; health insurance premiums, claims, penetration. Shows public and private payer demand and growth by geography.	Primary: programme administrative data and regulator statistics.
Q1	1. Government / regulator / statutory body	Ministry of Finance; NITI Aayog	Economic Survey (health chapter); Union Budget expenditure documents for MoHFW; NITI Aayog health sector papers	Policy intent, public spending trajectory, scheme allocations. Official but forward-looking statements are intentions, not outcomes.	Primary for allocations; secondary for NITI Aayog analyses.
Q1	2. Company filings and disclosures	Listed peers (NSE / BSE filings); target data room	Annual reports, investor presentations, earnings call transcripts; target management accounts and CIM	Company-reported volumes, occupancy, ARPOB, prescription share, installed base, capacity pipeline, capex; market commentary by management. Audited segments are strong; management market-share claims are not independent.	Primary for reported company metrics; secondary where management summarises the market.
Q1	3. International / multilateral	WHO; World Bank; UN DESA Population Division	WHO Global Health Expenditure Database and India country data; World Bank World Development Indicators; World Population Prospects	Cross-country benchmarks (spend per capita, public share), demographic and ageing projections. Harmonised from national sources, so useful for comparison and triangulation.	Secondary-compiled: authoritative harmonisation of national primary data.
Q1	4. Research institution / industry report	IQVIA; NATHEALTH, IPA, AIMED, AdvaMed, NASSCOM; ICRIER, NCAER, PHFI; established advisors (Bain, McKinsey, BCG, EY, KPMG, Deloitte)	Pharma market audits; industry association reports; peer-reviewed or institute studies on utilisation and costs; sector reports from established advisory firms	Segment sizing, growth forecasts, share by company and therapy area, city-tier splits. Check methodology, base year and whether the report was commissioned. PwC publications can be used but should not be the only source for a PwC-led analysis.	Secondary: analyses built on primary data; use only where methodology and data source are disclosed.
Q1	5. Reputable financial press	Reuters, Bloomberg, Financial Times, Business Standard, Mint, Economic Times	Dated news on capacity announcements, demand trends, scheme changes	Leads and timeliness only; each fact must be traced to a filing, official release or statistic.	Secondary.

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q2. Competitive landscape, consolidation, M&A and private equity activity (sector-level and target-specific)					
Q2	1. Government / regulator / statutory body	Competition Commission of India	Combination notices, orders and summaries published on the CCI site; Competition Act 2002 as amended 2023 and Combination Regulations	Precedent combinations in healthcare, relevant market definitions, remedies, overlaps, deal-value threshold application. Directly informs approval risk and consolidation limits.	Primary: regulatory orders and legal texts.
Q2	1. Government / regulator / statutory body	SEBI; NSE and BSE	Open offer letters under the Takeover Regulations; DRHP / RHP documents; LODR event disclosures; shareholding patterns; block and bulk deal data	Deal price, structure, selling shareholders, peer financials and valuation reasoning, ownership changes. Note: industry sections of offer documents are often commissioned reports.	Primary for transaction facts; the embedded industry reports are secondary.
Q2	1. Government / regulator / statutory body	Ministry of Corporate Affairs (MCA21 registry); DPIIT; RBI	Company filings (financial statements, charges, annual returns); DPIIT FDI inflow factsheets by sector (drugs and pharmaceuticals, medical and surgical appliances, hospital and diagnostic centres); RBI FDI reporting data	Private competitor financials and charges; aggregate foreign investment inflows by sector. Aggregate FDI data does not show deal-level terms.	Primary: statutory filings and official statistics.
Q2	2. Company filings and disclosures	Acquirers and peers	Notes on business combinations in annual reports (Ind AS 103 disclosures); press releases and stock exchange announcements of acquisitions	Consideration paid, goodwill, fair values, revenue of acquired entity, allowing implied multiples. Audited and comparable.	Primary.
Q2	2. Company filings and disclosures	Credit rating agencies (CRISIL, ICRA, CARE, India Ratings)	Sector outlook reports and company rating rationales	Occupancy, leverage, receivable days, payer mix and competitor financial strength. Independent but built from company data.	Secondary: analysis by regulated rating agencies.
Q2	4. Research institution / industry report	IVCA; Bain & Company; Grant Thornton Bharat; Venture Intelligence; commercial deal databases (PitchBook, Mergermarket, LSEG)	Annual private equity and M&A reviews; deal trackers; database exports	Deal volumes and values, investor activity, exits, holding periods. Use to locate transactions, then verify each against filings; undisclosed values in databases are often estimates.	Secondary: compilations; verify against primary filings.
Q2	5. Reputable financial press	Reuters, Bloomberg, Financial Times, Business Standard, Mint	Deal reports and bid rumours	Early signals of rival bidders and process dynamics. Treat unconfirmed reports as leads.	Secondary.
Q3. Licences, registrations, approvals and enforcement history (target-specific, regulatory framework sector-level)					
Q3	1. Government / regulator / statutory body	CDSCO; State drug and food regulators; State health departments	Drugs and Cosmetics Act and Rules; Medical Devices Rules 2017; licensing registers and SUGAM records; monthly not-of-standard-quality and spurious drug alerts; recall notices; clinical establishment registers	Validity, scope and transferability of manufacturing, sale, import and device licences; quality alerts and recalls tied to target products; inspection outcomes. Statutory and verifiable by licence number.	Primary: licence registers and regulatory texts.
Q3	1. Government / regulator / statutory body	Quality Council of India (NABH, NABL); National Medical Commission and state medical councils; AERB; state pollution control boards	Accreditation status listings; practitioner registers; radiation licences; biomedical waste authorisations	Facility and personnel accreditation and authorisation status; lapses. Verify each facility.	Primary.
Q3	1. Government / regulator / statutory body	Gazette of India; Ministry of Law and Justice (statute repository); Parliament	Notifications, rules, draft rules; parliamentary questions and answers on enforcement	Current legal text and amendment history; enforcement statistics reported to Parliament.	Primary.

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q3	1. Government / regulator / statutory body	Courts and tribunals (Supreme Court, High Courts, NCLT, ITAT, consumer commissions)	Cause lists and judgments; consumer commission orders	Litigation exposure, negligence rulings, regulatory challenges. Search party and promoter names.	Primary.
Q3	2. Company filings and disclosures	Target and peers	Regulatory correspondence, inspection reports, CAPA files, show-cause notices, annual report contingent liabilities, DRHP litigation sections	Closure status of observations; quantified contingent liabilities. Management files may omit; cross-check with regulator records.	Primary but self-reported; verify independently.
Q3	3. International / multilateral	US FDA; EU / EMA (EudraGMDP); WHO; UK MHRA	FDA Warning Letters, Form 483 observations, Establishment Inspection Reports, import alerts; GMP certificates; WHO prequalification and GMP listings	Export-market regulatory standing of Indian sites; repeat observations indicate systemic quality problems and valuation risk.	Primary: foreign regulator records.
Q3	4. Research institution / industry report	Law firms; regulatory consultants	Regulatory opinions and change-of-control analyses	Interpretation of licence continuity on change of control. Only as interpretation of primary texts.	Secondary: legal analysis.
Q4. Price controls, reimbursement rates and transaction approvals (sector-level policy, target-specific exposure)					
Q4	1. Government / regulator / statutory body	NPPA; Department of Pharmaceuticals; MoHFW	DPCO 2013; ceiling price notifications; price monitoring reports; National List of Essential Medicines; device price notifications; trade-margin orders	Which target products are price-controlled, ceiling price history, impact on margins. Map each target SKU to the notified lists.	Primary: legal orders and notified prices.
Q4	1. Government / regulator / statutory body	National Health Authority; CGHS; ESIC; state insurance agencies	PM-JAY Health Benefit Package rates; CGHS rate lists; state scheme rate cards; payment and claim rejection rules	Reimbursement rates by procedure, revision history, claim processing rules. Directly drives hospital and device-related revenue.	Primary.
Q4	1. Government / regulator / statutory body	DPIIT; RBI; MCA; CCI; SEBI	Consolidated FDI Policy; Press Notes including Press Note 3 of 2020; FEMA non-debt instrument rules and RBI master directions; Competition Act and Combination Regulations; Takeover Regulations	Permitted investment route and conditions by subsector, reporting duties, approvals, timeline. Use the current text only; policy changes often.	Primary: legal texts.
Q4	1. Government / regulator / statutory body	CBIC; GST Council; Central Board of Direct Taxes	GST rate notifications for medicines, devices and services; tax circulars	Tax effects on pricing and margin; classification disputes.	Primary.
Q4	2. Company filings and disclosures	Target	Product-wise price lists, contracts with payers and tenders, TPA agreements	Realised versus ceiling prices; contractual rate escalation or reset clauses.	Primary (target-specific).
Q4	4. Research institution / industry report	Law firms; tax advisors	Client alerts and transaction structuring notes	Pointers to relevant changes; every point must be confirmed against the primary notification.	Secondary.
Q5. Technology and digital health adoption, interoperability and integration (sector-level adoption, target-specific maturity)					
Q5	1. Government / regulator / statutory body	National Health Authority (ABDM); MoHFW	ABDM dashboards (ABHA creation, registered facilities and professionals, linked records); ABDM technical specifications and sandbox; National Digital Health Blueprint; EHR Standards for India; eSanjeevani telemedicine statistics	Level of national digital rails adoption, interoperability requirements and gaps, teleconsultation volumes. Official operational data.	Primary.

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q5	1. Government / regulator / statutory body	National Medical Commission / MoHFW; MeitY; TRAI	Telemedicine Practice Guidelines (2020); Digital India and India Stack documents; telecom subscription statistics	Legal basis for remote care; connectivity and digital readiness by region.	Primary.
Q5	2. Company filings and disclosures	Target and listed peers	Annual report technology sections, IT capex, investor presentations on EMR / ERP rollouts; BRSR (data privacy and cybersecurity indicators); technical diligence data room	Scale and ROI of digital programmes, application inventory, vendor dependencies, incident disclosures.	Primary (company-reported).
Q5	3. International / multilateral	WHO; World Bank; ITU; HL7 International; SNOMED International; ISO / IEC	WHO Global Strategy on Digital Health and any successor; World Bank digital development materials; HL7 FHIR, SNOMED CT and LOINC specifications; ISO health informatics standards	Benchmarks for maturity and standards the target should meet; basis for interoperability testing.	Primary for standards texts; secondary for strategy and benchmarking papers.
Q5	4. Research institution / industry report	NASSCOM; IAMAI; HIMSS; peer-reviewed journals; independent IT diligence provider	Digital health industry reports; adoption model assessments; academic studies on EMR adoption in India; independent technical assessments	Adoption benchmarks, vendor landscape, implementation lessons. Independent technical diligence is primary evidence about the target.	Secondary, except bespoke technical diligence on the target.
Q6. AI adoption, AI governance, responsible AI and clinical safety (sector-level frameworks, target-specific practice)					
Q6	1. Government / regulator / statutory body	MeitY / IndiaAI Mission; NITI Aayog	National AI governance guidance issued by MeitY / IndiaAI (confirm current version); IndiaAI programme documents; NITI Aayog National Strategy for AI (2018) and responsible AI approach papers (2021); healthcare AI initiatives under national programmes (confirm)	Direction of national AI policy, responsible-AI principles, adoption initiatives and datasets. Mostly principles and guidance, not binding law; treat as signals of future regulation.	Primary for official policy; guidance is non-binding.
Q6	1. Government / regulator / statutory body	ICMR	Ethical Guidelines for Application of Artificial Intelligence in Biomedical Research and Healthcare (2023); National Ethical Guidelines for Biomedical and Health Research Involving Human Participants (2017)	Governance expectations: accountability, validation, bias, transparency, consent, ethics committee review. Best India-specific yardstick for the target's AI governance.	Primary: official guidelines (non-statutory but widely referenced).
Q6	1. Government / regulator / statutory body	CDSCO	Medical Devices Rules 2017 and risk classification lists; any CDSCO guidance on software as a medical device (confirm whether draft or final); licensing registers	Whether each AI diagnostic or decision-support product is a regulated device, its class, licence number and quality-system evidence. Absence of licence is a deal-risk indicator.	Primary: legal texts and registers.
Q6	1. Government / regulator / statutory body	National Medical Commission / MoHFW; IRDAI	Telemedicine Practice Guidelines (role of AI and limits on prescribing); IRDAI regulatory sandbox and information and cyber security guidelines for insurers	Limits on AI in clinical decisions and insurance operations; human oversight requirements.	Primary.
Q6	2. Company filings and disclosures	Target and listed peers	AI use-case register, model documentation, validation reports, vendor contracts, patent filings (Indian Patent Office, WIPO PATENTSCOPE), annual report and DRHP claims on AI	Hype test: compare marketing claims to licences, validation evidence, patents, revenue attributable to AI, and customers who pay for it.	Primary (target-specific); claims need independent verification.

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q6	3. International / multilateral	WHO; OECD; UNESCO; IMDRF; ISO / IEC; NIST	WHO Ethics and Governance of AI for Health (2021) and guidance on large multimodal models (2024); OECD AI Principles; UNESCO Recommendation on the Ethics of AI (2021); IMDRF software-as-medical-device and good machine learning practice documents; ISO / IEC 42001 and 23894; IEC 62304 and ISO 14971; NIST AI Risk Management Framework	International yardsticks for governance maturity: oversight, explainability, bias, monitoring, lifecycle control, risk management. Useful for the acquirer's group-wide standard and for benchmarking the target.	Primary for standards and principles texts; they are not Indian law.
Q6	3. International / multilateral	EU institutions; US FDA; UK MHRA	EU AI Act (Regulation (EU) 2024/1689) with its high-risk provisions and phased application; US FDA guidance on AI-enabled device software and predetermined change control plans, and FDA list of AI-enabled devices	Applies if the target's products or data will be used within the acquirer's global business; shows direction of regulation and evidence expected for clinical AI.	Primary: foreign legal and regulatory texts.
Q6	4. Research institution / industry report	Peer-reviewed journals; ICMR and AIIMS-type institutions; Data Security Council of India; think tanks (for example Vidhi, Takshashila, ORF)	Published clinical validation studies on Indian populations; analyses of AI governance gaps; trial registry entries (CTRI)	Independent clinical performance and bias evidence; policy analysis. Advocacy groups may take a position; note it.	Secondary; peer-reviewed validation is strong evidence if relevant to the target's product.
Q6	5. Reputable financial press	Reuters, Bloomberg, Financial Times, Business Standard	Reports on AI deployments, product launches and incidents	Leads on adoption; confirm with filings or regulators.	Secondary.
Q7. Data governance, DPDP Act and DPDP Rules 2025, consent, localisation, cross-border flows and cybersecurity (sector-level law, target-specific compliance)					
Q7	1. Government / regulator / statutory body	MeitY; Data Protection Board of India	Digital Personal Data Protection Act 2023 (Gazette text); DPDP Rules 2025 (Gazette notification) and commencement notifications; government notices under the Act (for example on restricted countries or Significant Data Fiduciaries) if issued; explanatory material or FAQs; Board orders and advisories once issued	Binding duties: notice and consent, purpose limitation, security safeguards, breach intimation, retention and erasure, children's data and healthcare exemption, Significant Data Fiduciary duties, cross-border conditions, penalties. Commencement dates decide what the target must comply with at closing. Read the Gazette text, not summaries.	Primary: statute and subordinate legislation.
Q7	1. Government / regulator / statutory body	MeitY; Ministry of Law and Justice	IT Act 2000 (including s.43A) and SPDI Rules 2011 (relevant until replaced under the commencement schedule); Supreme Court judgment in Justice K.S. Puttaswamy v Union of India (2017)	Current-law baseline for the transition period and constitutional privacy standard.	Primary.
Q7	1. Government / regulator / statutory body	National Health Authority; MoHFW	ABDM Health Data Management Policy; ABDM consent and health information exchange specifications; draft Digital Information Security in Healthcare Act 2018 (draft only, not enacted)	Health-specific consent, data-sharing and security expectations for ABDM-connected providers; do not treat the 2018 draft as law.	Primary (policy and specifications).
Q7	1. Government / regulator / statutory body	CERT-In; NCIIPC; IRDAI	CERT-In Directions of 28 April 2022 under the IT Act (incident reporting within six hours, log retention); CERT-In advisories and annual reports; IRDAI information and cyber security guidelines for insurers	Mandatory incident reporting and logging; sector threat patterns. Compare the target's incident log and reporting practice.	Primary.

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q7	1. Government / regulator / statutory body	ICMR; Department of Biotechnology; CDSCO	ICMR ethical guidelines (consent for research and secondary use); Biological Data Storage, Access and Sharing Policy (confirm status); New Drugs and Clinical Trials Rules 2019; CTRI	Rules for research use, data sharing and any restrictions on moving health or biological data abroad.	Primary.
Q7	2. Company filings and disclosures	Target and listed peers	Privacy notices and consent forms; data inventory; processor contracts; penetration test and audit reports; breach register; BRSR disclosures on data privacy complaints and breaches; stock exchange cyber incident disclosures; annual report risk factors	Evidence of actual practice, incident history and disclosure, to test against DPDP Rules 2025 duties. Most decision-relevant evidence for target risk, but self-reported.	Primary (target-specific); independently test by technical audit.
Q7	3. International / multilateral	OECD; WHO; ISO / IEC; EU and US authorities	OECD Recommendation on Health Data Governance (2016); ISO / IEC 27001, 27701, 27799; GDPR and HIPAA texts	Benchmarks for governance and the acquirer's group requirements on transfers into the group.	Primary for texts; international benchmarks, not Indian law.
Q7	4. Research institution / industry report	Data Security Council of India; IBM (Cost of a Data Breach Report, India data); peer-reviewed studies; law firms	DSCI privacy and security frameworks and reports; breach cost study; analyses of the DPDP Rules	Industry practice benchmarks and breach cost context. Law firm commentary only as a pointer to the Gazette text.	Secondary.
Q7	5. Reputable financial press	Reuters, Bloomberg, Business Standard, Mint	Reports on specific healthcare data breaches or enforcement	Identify incidents to verify via CERT-In records, parliamentary answers or exchange filings.	Secondary.
Q8. Operational risk: patient safety, quality, supply chain, workforce and continuity (target-specific, with sector-wide pressures)					
Q8	1. Government / regulator / statutory body	CDSCO; Indian Pharmacopoeia Commission; State regulators	Not-of-standard-quality alerts; recall notices; Pharmacovigilance Programme of India and Materiovigilance Programme of India outputs	Quality and adverse event signals linked to target products and manufacturers.	Primary.
Q8	1. Government / regulator / statutory body	Quality Council of India (NABH); consumer commissions; medical councils	Accreditation reports and standards; negligence and consumer orders; disciplinary orders	Clinical quality, complaint and negligence patterns for hospital and care assets.	Primary.
Q8	1. Government / regulator / statutory body	DGCI&S; Department of Pharmaceuticals; Parliamentary Standing Committees	Import data by HS code and origin for APIs, intermediates and device components; PLI scheme documents; committee reports on pharmaceutical supply dependence	Quantified dependence on imports and concentration by country; policy support for localisation.	Primary.
Q8	1. Government / regulator / statutory body	MoSPI; National Medical Commission; Indian Nursing Council; MoHFW	Periodic Labour Force Survey; practitioner registries; Rural Health Statistics	Workforce supply, wages, density by state; attrition context.	Primary.
Q8	2. Company filings and disclosures	Target and listed peers	BRSR (employee turnover, safety, product recalls); auditor reports (key audit matters, CARO); risk management committee reports; plant and site quality records; insurance policies	Reported recalls, turnover, safety, contingent liabilities, site concentration and insurance cover.	Primary (company-reported); verify with inspections and site visits.
Q8	3. International / multilateral	WHO; World Bank; UN Comtrade; WTO; US trade authorities (for exporters)	Global health workforce statistics; trade data; tariff notices	Cross-checks of trade dependence and workforce benchmarks; export tariff and trade policy exposure.	Primary for trade notices and data; secondary for compiled analysis.

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q8	4. Research institution / industry report	PHFI; peer-reviewed journals; rating agencies	Studies on quality of care and workforce; sector credit reports	Benchmarks for quality outcomes and staffing; independent view of operating resilience.	Secondary.
Q9. Revenue quality, payer exposure, contract durability and valuation (target-specific, sector benchmarks)					
Q9	1. Government / regulator / statutory body	MCA; Income Tax Department and ITAT; GSTN / CBIC	Statutory financial statements and annual returns; tax assessment orders; GST registrations and notices	Verified financials, tax disputes, related-party disclosures, charges and debt. Foundation of the quality-of-earnings review.	Primary.
Q9	1. Government / regulator / statutory body	National Health Authority; CGHS; ESIC; IRDAI; Comptroller and Auditor General	PM-JAY claims and rates; scheme payment practices; IRDAI claims statistics; CAG performance audits of health schemes (confirm which are available)	Payment delays, rejection rates, rate resets, insurer and TPA concentration. Core inputs to revenue quality and working capital.	Primary.
Q9	1. Government / regulator / statutory body	RBI; Ministry of Finance	Handbook of Statistics on the Indian Economy; government bond yields; exchange rate series	Rates, inflation, currency inputs for DCF and financing assumptions.	Primary.
Q9	2. Company filings and disclosures	Target; listed peers on NSE / BSE	Audited financials with notes (related parties, contingencies, segments, receivables ageing); investor presentations; earnings call transcripts; share price data	Peer margins, working capital, payer mix, valuation multiples from audited numbers.	Primary for reported numbers; multiples are derived by us.
Q9	2. Company filings and disclosures	Target counterparties	Customer, payer, distributor and tender contracts	Tenor, renewal, termination, change-of-control, price reset and exclusivity terms; durability of revenue.	Primary (target-specific).
Q9	3. International / multilateral	IMF; World Bank	World Economic Outlook; Article IV consultations; India economic updates	Macro scenario inputs (growth, inflation, currency).	Secondary: forecasts and analysis.
Q9	4. Research institution / industry report	Credit rating agencies; Bain, Grant Thornton Bharat, IVCA; academic datasets (for example country risk premium data from NYU Stern)	Rating rationales; PE and M&A reviews; valuation datasets	Independent view of credit quality, deal multiples and discount-rate inputs. Verify the underlying data.	Secondary.
Q9	5. Reputable financial press	Reuters, Bloomberg, Financial Times, Mint, Business Standard	Valuation and deal reporting	Context and leads; confirm with filings.	Secondary.
Q10. Emerging threats, future trends and scenarios (sector-level, tested against target sensitivities)					
Q10	1. Government / regulator / statutory body	MoHFW; CDSCO; Department of Pharmaceuticals; MeitY; NITI Aayog	Draft rules and consultation notices in the Gazette; successor to the draft Drugs, Medical Devices and Cosmetics Bill (confirm status); AI and DPDP policy announcements	Pipeline of changes to pricing, device and software regulation, data protection and AI rules. Upcoming rules are scenario triggers.	Primary (official drafts); outcomes uncertain.
Q10	1. Government / regulator / statutory body	Parliament; Parliamentary Standing Committee on Health and Family Welfare; Union Budget	Committee reports; Question Hour answers; budget speeches	Signals on policy direction and enforcement intent.	Primary.
Q10	2. Company filings and disclosures	Listed peers and acquirers	Management outlook and risk factor sections; capex and capacity plans	How informed incumbents see disruption and where they invest.	Primary for stated plans; opinion is not independent evidence.

Research question	Preferred source type	Example source / organization	Specific document or source to look for	Why it is useful (data type, reliability, impact)	Primary or secondary evidence (with rationale)
Q10	3. International / multilateral	WHO; OECD (including OECD.AI policy observatory); World Bank; IMF; EU	WHO guidance and global health trends; OECD AI policy tracking; EU AI Act implementation timeline	Global regulatory and technology direction relevant to the acquirer's group.	Primary for official texts; secondary for analyses.
Q10	4. Research institution / industry report	Peer-reviewed journals; ICRIER, NCAER, ORF; McKinsey, BCG, Bain and other established advisors; industry bodies	Studies on technology, new care models, insurance expansion and competition	Hypotheses on disruptors to test with data. Treat forecasts as scenarios, not facts.	Secondary.
Q10	4. Research institution / industry report	Own structured expert and customer interviews	Interviews with regulatory counsel, clinicians, payers, former executives	Tests assumptions that documents cannot; must follow the engagement data governance rules.	Primary (first-hand), but individual opinion; triangulate.
Q10	5. Reputable financial press	Reuters, Bloomberg, Financial Times, Business Standard	Reports on tariffs, supply shocks, new entrants and regulatory moves	Early warning, to be confirmed with official sources.	Secondary.

AI and data governance: evidence checklist

This expands Q6 and Q7 along the five areas you specified, so that each can be assigned to a workstream.

Area	Authoritative sources to read	Evidence to extract from the target
AI regulatory frameworks, governance and healthcare AI safety	MeitY / IndiaAI guidance; ICMR 2023 AI guidelines; NITI Aayog responsible AI papers; WHO 2021 and 2024 guidance; OECD AI Principles; EU AI Act; NIST AI RMF; ISO / IEC 42001	Governance structure, risk classification, accountability owners, incident and safety reporting; which framework the target claims to follow and proof of operation.
Data protection, patient consent, privacy, localisation and cross-border	DPDP Act 2023 and DPDP Rules 2025 (Gazette); SPDI Rules during transition; ABDM Health Data Management Policy and consent specifications; ICMR consent guidelines; DBT biological data policy (confirm); sectoral rules	Notice and consent artefacts, purpose limits, retention, children's data and healthcare exemption scope, processors, Significant Data Fiduciary risk, offshore flows and any government restrictions or sectoral localisation.
Cybersecurity, interoperability and clinical validation	CERT-In Directions 2022 and advisories; IRDAI cyber guidelines; ISO / IEC 27001 and 27799; ABDM specifications; HL7 FHIR and SNOMED CT; CDSCO device rules; CTRI; peer-reviewed validation	Controls and incident history, standards conformance, evidence that AI performance was validated on Indian patients and re-validated after changes.
Algorithmic accountability, transparency and explainability, bias and human oversight	ICMR 2023; WHO 2021; OECD; UNESCO; NIST AI RMF; ISO / IEC 23894; EU AI Act high-risk requirements; IMDRF and FDA good machine learning practice; DPDP Rules 2025 duties for Significant Data Fiduciaries (algorithmic due diligence)	Model documentation, subgroup performance and bias tests, explainability method, clinician override logs, post-deployment monitoring, named accountability.
Regulatory treatment of AI-enabled medical technologies and organisations deploying AI	Medical Devices Rules 2017 and CDSCO guidance on device software; Telemedicine Practice Guidelines; IRDAI rules for insurers; NABH standards; FDA, EU MDR / IVDR and AI Act, IMDRF	Device status and class, licence evidence, permitted roles for AI in care and claims, and the regulatory path for products the acquirer wants to deploy in or out of India.

Triangulation plan

Data point	Primary anchor	Cross-check	Reconciliation rule
Health expenditure and financing mix	MoHFW National Health Accounts; NSO surveys	WHO Global Health Expenditure Database; World Bank	Match definitions (current health expenditure, out-of-pocket share). Differences usually come from method and year; use official series and explain the gap.

Data point	Primary anchor	Cross-check	Reconciliation rule
Subsector revenue pool and growth	Listed peer filings; NSO ASI; DPIIT and trade data	IQVIA and industry association reports; advisor reports	Rebuild bottom-up from filings and official data; accept third-party figures only when the base year and definitions reconcile.
Hospital capacity and utilisation	Rural Health Statistics; National Health Profile; state registers; company filings	NABH lists; PM-JAY empanelment; NSS utilisation	Reconcile beds, occupancy and patient volumes by geography; flag inconsistent definitions.
Import dependence and supply risk	DGCI&S trade data; Department of Pharmaceuticals	UN Comtrade; Parliamentary committee reports; target BoM	Match HS codes to target inputs; weight by target spend, not national averages.
Transaction multiples and valuation	Offer letters, DRHPs, Ind AS 103 notes, exchange disclosures	Deal databases; Bain, Grant Thornton Bharat, IVCA	Use disclosed values only for the core set; mark estimated values separately.
Payer mix and reimbursement	Target contracts and ledgers; PM-JAY, CGHS, IRDAI data	Rating rationales; CAG audits; peer disclosures	Test target data against scheme-level payment and rejection patterns.
AI adoption and capability	Target AI register, licences, validation reports, patents	Peer-reviewed studies; ICMR and IndiaAI materials; customer interviews	Count only deployed, validated and paid-for use cases; discount claims lacking regulatory or clinical evidence.
Data protection posture and incidents	Target breach register, audits, notices; CERT-In records; BRSR	Parliamentary answers; exchange disclosures; independent penetration test	Compare self-reported incident history with external records; discrepancies are a red flag.
Regulatory status of the target	CDSCO and state registers; NABH lists; foreign regulator databases	Counsel opinion; court records	A licence counts only if verified against the issuing authority's record.

Source handling rules

- Log every source: organisation, document title as shown on the document, edition or version, publication date, date accessed, location, and what number or statement was taken. Do not cite from memory.
- Use the Gazette text for any law, rule or notification. Treat law firm and press summaries as pointers only.
- The document names organizations and document types we expect to exist. We have not verified current availability, editions or URLs in this session. Where a title is marked "confirm", locate it before use; if it cannot be found, drop it rather than substitute a guess.
- For commissioned industry reports (including those in offer documents), identify the commissioning party and methodology. Do not treat them as independent.
- Where two sources disagree, report both, state the reason (definition, year, scope), and use the one closest to the original data. Only a primary source can settle a target-specific fact.
- Exclude blogs, SEO articles, anonymous summaries and AI-generated content, unless they link directly to a primary source that we then read ourselves.
- DPDP: all privacy-related evidence requests in this exercise should be framed under the DPDP Act and DPDP Rules 2025, with commencement dates confirmed by counsel. Request anonymised or aggregated target data wherever possible and avoid collecting patient-level personal data during diligence.
- PwC internal data governance: the policy has not been provided in this session, so storage, access, retention and tool-use rules for sourced material must be checked against it before collection begins.

Link register (organization home pages)

Organization	Tier	Home page
ABDM	Government / regulator	https://abdm.gov.in
AERB	Government / regulator	https://aerb.gov.in
BSE	Government / regulator	https://www.bseindia.com
CBIC	Government / regulator	https://www.cbic.gov.in
CCI	Government / regulator	https://cci.gov.in
CDSCO	Government / regulator	https://cdsco.gov.in
Central Board of Direct Taxes	Government / regulator	https://www.incometax.gov.in
Central Bureau of Health Intelligence	Government / regulator	https://cbhidghs.mohfw.gov.in
CERT-In	Government / regulator	https://www.cert-in.org.in
CGHS	Government / regulator	https://cghs.gov.in
Competition Commission of India	Government / regulator	https://cci.gov.in
Comptroller and Auditor General	Government / regulator	https://cag.gov.in
Courts and tribunals	Government / regulator	https://ecourts.gov.in
Department of Biotechnology	Government / regulator	https://dbtindia.gov.in
Department of Commerce	Government / regulator	https://commerce.gov.in
Department of Pharmaceuticals	Government / regulator	https://pharmaceuticals.gov.in
DGCI&S	Government / regulator	https://tradestat.commerce.gov.in
DPIIT	Government / regulator	https://dpiit.gov.in
ESIC	Government / regulator	https://www.esic.gov.in
Gazette of India	Government / regulator	https://egazette.gov.in
GST Council	Government / regulator	https://gstcouncil.gov.in
ICMR	Government / regulator	https://www.icmr.gov.in
IIPS	Government / regulator	https://www.iipsindia.ac.in
Income Tax Department	Government / regulator	https://www.incometax.gov.in
IndiaAI Mission	Government / regulator	https://indiaai.gov.in
Indian Nursing Council	Government / regulator	https://indiannursingcouncil.org
Indian Pharmacopoeia Commission	Government / regulator	https://ipc.gov.in

Organization	Tier	Home page
IRDAI	Government / regulator	https://irdai.gov.in
MCA	Government / regulator	https://www.mca.gov.in
MeitY	Government / regulator	https://www.meity.gov.in
Ministry of Corporate Affairs	Government / regulator	https://www.mca.gov.in
Ministry of Finance	Government / regulator	https://finmin.nic.in
Ministry of Law and Justice	Government / regulator	https://lawmin.gov.in
MoHFW	Government / regulator	https://mohfw.gov.in
MoSPI	Government / regulator	https://mospi.gov.in
NABH	Government / regulator	https://nabh.co
NABL	Government / regulator	https://nabl-india.org
National Health Authority	Government / regulator	https://nha.gov.in
National Health Systems Resource Centre	Government / regulator	https://nhsrc.gov.in
National Medical Commission	Government / regulator	https://www.nmc.org.in
National Statistical Office	Government / regulator	https://mospi.gov.in
NCIIPC	Government / regulator	https://nciipc.gov.in
NITI Aayog	Government / regulator	https://niti.gov.in
NPPA	Government / regulator	https://nppaindia.nic.in
NSE	Government / regulator	https://www.nseindia.com
Parliament	Government / regulator	https://sansad.in
Parliamentary Standing Committee on Health and Family Welfare	Government / regulator	https://sansad.in
Parliamentary Standing Committees	Government / regulator	https://sansad.in
Pharmexcil	Government / regulator	https://pharmexcil.com
Quality Council of India	Government / regulator	https://qci.org.in
RBI	Government / regulator	https://www.rbi.org.in
Registrar General of India	Government / regulator	https://censusindia.gov.in
SEBI	Government / regulator	https://www.sebi.gov.in
State drug and food regulators	Government / regulator	https://cdsco.gov.in
Supreme Court	Government / regulator	https://www.sci.gov.in

Organization	Tier	Home page
TRAI	Government / regulator	https://www.trai.gov.in
EU	International / multilateral	https://eur-lex.europa.eu
EU / EMA	International / multilateral	https://www.ema.europa.eu
EU institutions	International / multilateral	https://eur-lex.europa.eu
HL7 International	International / multilateral	https://www.hl7.org
IMDRF	International / multilateral	https://www.imdrf.org
IMF	International / multilateral	https://www.imf.org
ISO / IEC	International / multilateral	https://www.iso.org
ITU	International / multilateral	https://www.itu.int
NIST	International / multilateral	https://www.nist.gov/itl/ai-risk-management-framework
OECD	International / multilateral	https://www.oecd.org
OECD.AI	International / multilateral	https://oecd.ai
SNOMED International	International / multilateral	https://www.snomed.org
UK MHRA	International / multilateral	https://www.gov.uk/government/organisations/medicines-and-healthcare-products-regulatory-agency
UN Comtrade	International / multilateral	https://comtradeplus.un.org
UN DESA Population Division	International / multilateral	https://population.un.org/wpp
UNESCO	International / multilateral	https://www.unesco.org
US FDA	International / multilateral	https://www.fda.gov
US trade authorities	International / multilateral	https://www.ustr.gov
WHO	International / multilateral	https://www.who.int
World Bank	International / multilateral	https://data.worldbank.org
WTO	International / multilateral	https://www.wto.org
AdvaMed	Research / industry	https://www.advamed.org
AIMED	Research / industry	https://aimedindia.com
Bain	Research / industry	https://www.bain.com
Bain & Company	Research / industry	https://www.bain.com
BCG	Research / industry	https://www.bcg.com
CARE	Research / industry	https://www.careratings.com

Organization	Tier	Home page
CRISIL	Research / industry	https://www.crisilratings.com
Data Security Council of India	Research / industry	https://www.dsci.in
Deloitte	Research / industry	https://www.deloitte.com
EY	Research / industry	https://www.ey.com
Grant Thornton Bharat	Research / industry	https://www.grantthornton.in
HIMSS	Research / industry	https://www.himss.org
IAMAI	Research / industry	https://www.iamai.in
IBM	Research / industry	https://www.ibm.com/reports/data-breach
ICRA	Research / industry	https://www.icra.in
ICRIER	Research / industry	https://icrier.org
India Ratings	Research / industry	https://www.indiaratings.co.in
IPA	Research / industry	https://www.ipa-india.org
IQVIA	Research / industry	https://www.iqvia.com
IVCA	Research / industry	https://www.ivca.in
KPMG	Research / industry	https://kpmg.com
LSEG	Research / industry	https://www.lseg.com
McKinsey	Research / industry	https://www.mckinsey.com
Mergermarket	Research / industry	https://www.mergermarket.com
NASSCOM	Research / industry	https://nasscom.in
NATHEALTH	Research / industry	https://nathealthindia.org
NCAER	Research / industry	https://www.ncaer.org
NYU Stern	Research / industry	https://pages.stern.nyu.edu/~adamodar
ORF	Research / industry	https://www.orfonline.org
PHFI	Research / industry	https://phfi.org
PitchBook	Research / industry	https://pitchbook.com
Takshashila	Research / industry	https://takshashila.org.in
Venture Intelligence	Research / industry	https://www.ventureintelligence.com
Vidhi	Research / industry	https://vidhilegalpolicy.in

Organization	Tier	Home page
Bloomberg	Financial press	https://www.bloomberg.com
Business Standard	Financial press	https://www.business-standard.com
Economic Times	Financial press	https://economictimes.indiatimes.com
Financial Times	Financial press	https://www.ft.com
Mint	Financial press	https://www.livemint.com
Reuters	Financial press	https://www.reuters.com